



NEOLIFE HOSPITAL PVT LTD.,

A Special Place For Your Little Miracle...

Tanig

20 Yrs 1m

8/4/18

wt 76 kg.

Inj RT 0.5 cc 1m stat.

Inj Rabies vaccine (Abhayrab)
0 dose - 8/4/18

Next due 11/4/18, 15/4, 22/4, 6/5

Review on :

Table 1. Guidance for the post exposure treatment based on WHO TRS 982 Recommendations.

Category of exposure	Type of exposure to a domestic or wild ^a animal suspected or confirmed to be rabid, or animal unavailable for testing	Recommended post-exposure prophylaxis
I	Touching or feeding animals, Licks on intact skin, Contact of intact skin with secretions or excretions of a rabid animal or human case	None, if reliable case history is available (in case of uncertainty vaccine to be administered)
II	Nibbling of uncovered skin, Minor scratches or abrasions without bleeding	Administer vaccine immediately. ^b Stop treatment if animal remains healthy throughout an observation period of 10 days ^c or is proven to be negative for rabies by a reliable laboratory using appropriate diagnostic techniques.
III	Single or multiple transdermal bites ^d or scratches, licks on broken skin. Contamination of mucous membrane with saliva (i.e. licks) Exposure to bats ^e	Administer rabies vaccine immediately, and rabies immunoglobulin, preferably as soon as possible after initiation of post-exposure prophylaxis. Rabies immunoglobulin can be injected up to 7 days after first vaccine dose administration. Stop treatment if animal remains healthy throughout an observation period of 10 days or is proven to be negative for rabies by a reliable laboratory using appropriate diagnostic techniques.

- ^a Exposure to rodents, rabbits or hares does not routinely require rabies post-exposure prophylaxis.
 - ^b If an apparently healthy dog or cat in or from a low-risk area is placed under observation, treatment may be delayed.
 - ^c This observation period applies only to dogs and cats. Except for threatened or endangered species, other domestic and wild animals suspected of being rabid should be euthanized and their tissues examined for the presence of rabies antigen by appropriate laboratory techniques.
 - ^d Bites especially on the head, neck, face, hands and genitals are Category III exposures because of the rich innervation of these areas.
 - ^e Post-exposure prophylaxis should be considered when contact between a human and a bat has occurred, unless the exposed person can rule out a bite or scratch or exposure of a mucous membrane.
- ^f Bat rabies has not been conclusively proved in India and hence exposure to bats does not warrant treatment.

REFERENCE:

1. http://www.ncdc.gov.in/Rabies_guidelines2014.pdf
2. WHO Technical Report Series 982, WHO EXPERT CONSULTATION ON RABIES.
3. http://www.who.int/vaccine_safety/initiative/tools/Rabies_Vaccine_rates_information_sheet.pdf

Warning :
To be sold by retail on the prescription of a Registered Medical Practitioner only.

Caution :
Do not use if any particulate matter is observed after reconstitution.

® Registered Trademark



Manufactured & Marketed by:

Human Biologicals Institute

(A division of Indian Immunologicals Limited)

Survey No.: 281-284 and 321, Biotech Park, Phase - III, Karkapatta Village, Markook Mandal, Siddipet (Dist.) - 502 281, Telangana, India.

RABIES VACCINE, HUMAN I.P.

(PURIFIED VERO CELL RABIES VACCINE)

Abhayrab®

For human use



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RABIES VACCINE, HUMAN I.P.

(PURIFIED VERO CELL RABIES VACCINE)

Abhayrab®

DESCRIPTION:

Abhayrab® is a lyophilized, purified, inactivated Rabies vaccine containing inactivated Rabies Virus (L.Pasteur 2061/Vero Strain propagated in Vero cells). Thiomersal is added as preservative. Rabies is an acute viral disease which causes fatal encephalomyelitis in virtually all the warm blooded animals including man. The virus can be found in the wild and some domestic animals and is transmitted through their saliva (i.e., bites, scratches, licks on broken skin and mucous membrane) to other animals and humans.

COMPOSITION:

Purified lyophilized Rabies antigen derived from Rabies virus (L. Pasteur 2061/ Vero Strain propagated in Vero Cells), Inactivated.

Potency: ≥ 2.5 I.U. per vial.

Stabilizers: Maltose and Human Albumin... q.s.

Preservative: Thiomersal $\leq 0.015\%$ w/v.

INDICATIONS:

a) Pre Exposure Vaccination (Prior to exposure to rabies virus):

This is indicated for all high risk groups of people such as:

- Laboratory staff handling the virus and infected material
- Clinicians and persons attending to human rabies cases
- Veterinarians
- Animal handlers and catchers
- Wildlife wardens
- Quarantine officers; and
- Travelers from rabies free areas to rabies endemic areas

b) Post Exposure Vaccination (after known/ suspected exposure to rabies virus):

This is indicated for:

- Persons after contact with/ bite by a suspected case of or a rabid animal.
- See Table 1 for WHO recommendations for course of action to be taken after contact with a suspected or confirmed rabid animal.

RECONSTITUTION:

Prior to use, reconstitute the lyophilized vaccine with 0.5 mL or 1.0 mL diluent as per the pack supplied. Any reconstituted vaccine should be used as soon as possible. Any left out amount must be stored in a refrigerator at $+2^{\circ}\text{C}$ to $+8^{\circ}\text{C}$ and used within 6 hours of reconstitution. If not maintained at $+2^{\circ}\text{C}$ to $+8^{\circ}\text{C}$, and/or stored for more than 6 hours, the vaccine must be immediately discarded.

DOSAGE AND ADMINISTRATION:

- For intramuscular: Reconstituted vaccine of 0.5 mL or 1.0 mL as per pack supplied to be administered by deep intramuscular route in the deltoid region in adults & in anterolateral aspect of thigh in children.

Vaccination Card

Name Sanj

Address _____

☐ Pre - Exposure Prophylaxis

☒ Post - Exposure Prophylaxis

Date of Bite 2/4/18

Bitten by (Specify the Animal)

street dog

Doctor's Signature

Important : Please carry this card every time you visit the doctor.

PLL087101/17

- **For intradermal:** In India, regulatory approval by the DCGI for intradermal administration of Abhayrab[®] has been restricted to anti-rabies clinics meeting the following criteria: the attendance is at least 10 cases of post-exposure treatment per day; there are adequately trained staff; the cold chain is maintained; adequate supply of suitable syringes and needles can be ensured; and the staff are well versed in the management of open vial and safe storage practices.

0.1 mL each of the reconstituted vaccine to be administered at two different lymphatic drainage sites, usually the left and right upper arm. Vaccine administered intradermally must raise a visible and palpable "bleb" in the skin. Aseptic technique and a separate sterile needle and syringe must be used to draw up vaccine for each patient and for each dose. The remainder can be used for another patient, provided that the vial is stored in a refrigerator at +2°C to +8°C and the total content is used within 6 hours.

IMMUNIZATION SCHEDULE:

For Intramuscular Vaccination:

- **Pre-Exposure:** 0.5 mL or 1.0 mL of reconstituted vaccine (as per diluent supplied) on days 0, 7 and 21 or 28. Only people whose occupation puts them at continual or frequent risk of exposure should receive periodic booster doses whenever the rabies virus neutralizing antibody titre drops below 0.5 IU/mL.
- **Post-Exposure:**
 - For a new case: 0.5 mL or 1.0 mL of reconstituted vaccine (as per diluent supplied) on days 0, 3, 7, 14 and 28.
 - Rabies immunoglobulin should be administered preferably as soon as possible after initiation of post-exposure prophylaxis in case of Category III bites but not beyond 7 days from the first dose of vaccination.
 - For previously immunized individuals: One dose (0.5 mL or 1.0 mL, as per diluent supplied) of vaccine intramuscularly at one site on day 0 and day 3. The administration of Rabies immunoglobulin is not required in such cases.
 - For immunocompromised individuals: They should definitely receive rabies immunoglobulin in addition to a full post-exposure vaccination series as listed above both in Category II and Category III bites.

For Intradermal Vaccination:

- **Pre-Exposure:** One dose of 0.1 mL of the reconstituted vaccine at one site on days 0, 7 and 21 or 28. Only people whose occupation puts them at continual or frequent risk of exposure should receive periodic booster doses whenever the rabies virus neutralizing antibody titre drops below 0.5 IU/mL.
- **Post exposure:**
 - For a new case: Updated two-site Thai Red Cross Regimen (2-2-0-0-2): 0.1 mL of the reconstituted vaccine at two different intradermal sites on each of days 0, 3, 7 and 28. Vaccine administered intradermally raises a visible and palpable "bleb" in the skin. Intradermal administration must be carried out by staff trained in this technique.
 - Rabies immunoglobulin should be administered preferably as soon as possible after initiation of post-exposure prophylaxis in case of Category III bites but not beyond 7 days from the first dose of vaccination.
 - For previously immunized individuals: One dose (0.1 mL) of vaccine intradermally at one site on day 0 and day 3. The administration of Rabies immunoglobulin is not required in such cases.
 - For immunocompromised individuals: Intradermal administration should not be used for immunocompromised individuals or individuals receiving chloroquine - based antimalarial treatment or long term corticosteroid or other immunosuppressive therapy.

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CONTRAINDICATIONS:

- In case of Pre-Exposure prophylaxis, rabies vaccination is contraindicated in severe febrile illness, acute or chronic progressive illness and known hypersensitivity to any of the components of the vaccine.
- As Rabies is a fatal disease, there are no contraindications in case of post exposure vaccination.

SPECIAL CONSIDERATIONS:

- ²In suspected post exposure, considering the severity and fatal implications of the disease, pregnancy and lactation are not contraindications for vaccination.

PRECAUTIONS:

- Concurrent use of immunosuppressive agents like corticosteroids should be avoided as it may hamper the development of protective antibodies.
- When anti-rabies immunoglobulin is recommended along with the first dose of rabies vaccination, it has to be administered with a different syringe & needle and at a site different and away from the site of vaccination.
- Intramuscular route must be used in cases of immunocompromised / immunosuppressed individuals and those on antimalarial medications. They should also receive RIG in Category II bites.
- Delay in the commencement of post - bite therapy, incomplete and irregular therapy can cause failure of vaccination and inadequate protection against rabies.
- Vaccine should never be administered into the gluteal region, where absorption is unpredictable.
- As with any injectable vaccine, hypersensitivity or anaphylaxis can occur with Abhayrab[®] and thus Inj. Adrenaline (1:1000) and other medications including anti-histaminics should be readily available during vaccination.
- Alcohol and other disinfecting agents must be allowed to evaporate from the skin before injection of the vaccine.
- The vaccine recipients should be kept under medical supervision for at least 30 minutes after vaccination to monitor for any undesirable effects.

WARNINGS:

- The vaccine should never be given by intravascular route.
- Same Syringe or site should not be used for administering the Immunoglobulin and the vaccine.
- Keep out of reach of Children.

ADVERSE EVENTS:

Local minor events: Mild pain, erythema, induration, pruritus, rash, oedema at the site of injection.
Systemic mild events: Mild fever, headache, myalgia, malaise, nausea, dizziness.
Very rarely: High fever, gastrointestinal symptoms, lymphadenopathy, erythema multiforme, arthritis and anaphylaxis.

SHELF LIFE:

36 months from the date of manufacture.

STORAGE AND TRANSPORTATION:

Store and transport the vaccine at +2°C to +8°C. Do not freeze.

PRESENTATION: Abhayrab[®] is available as

- Lyophilized vial with 0.5 mL or 1.0 mL sterile diluent I.P.
- Combo pack: Contains lyophilized vaccine vial, 0.5 mL or 1.0 mL of Sterile diluent I.P., Sterile disposable syringe with needle & Alcohol swab.

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Category of post exposure

I

II

III

Previous anti-rabies treatment

YES

NO

Completion Date:.....

Immunization schedule :

Administration type	Pre- Exposure	Post- Exposure		
		For New Case	For Previously Immunized Individuals	For Immuno- compromised Individuals
INTRAMUSCULAR	On 0, 7 & 21 or 28 th day.	On 0, 3, 7, 14 & 28 th day.	One dose of vaccine at one site on days 0 & 3.	Passive immunization with Rabies immunoglobulin along with Post exposure vaccination on 0, 3, 7, 14 & 28 th day in category II & III bites.
INTRADERMAL	On 0, 7 & 21 or 28 th day.	On 0, 3, 7 & 28 th day.	One dose of vaccine at one site on days 0 & 3.	ID vaccination not recommended. Follow IM regimen along with passive immunization.

RIG: YES / NO

ERIG / HRIG DOSAGE:

In case of Category III exposure RIG should be given on Day 0 as per WHO recommendation.

Record of vaccination schedule :

Administration type	No. of dose	Pre - Exposure	Post- Exposure			Vaccination Date	Batch No.	Next due date
			New case	Previously Immunized	Immuno-compromised *			
INTRAMUSCULAR	1 st	0 day	0 day	0 day	0 day	8/2/18		
	2 nd	7 th day	3 rd day	3 rd day	3 rd day			11/4/18
	3 rd	21 st / 28 th day	7 th day	—	7 th day			15/4/18
	4 th	—	14 th day	—	14 th day			22/4/18
	5 th	—	28 th day	—	28 th day			3/5/18
INTRADERMAL	1 st	0 day	0 day	0 day				
	2 nd	7 th day	3 rd day	3 rd day	**			
	3 rd	21 st / 28 th day	7 th day	—				
	4 th	—	28 th day	—				

Put Tick mark (✓) in the above provided boxes based on the nature of the case.

* For immunocompromised individuals they should definitely receive RIG along with post exposure vaccination schedule in Category II & III bites.
** ID vaccination not recommended. Follow IM regimen.